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UBAT DEMAM

MAL19940047XCZ

Pakfarloo **百花露退热露**
FEVER MIXTURE

Paracetamol 250mg / 5ml

主治：緩解發燒、頭痛和疼痛的症狀。

服法：6歲至12歲:每次服2茶匙 (10 ml)。

12歲以上和成人:每次服3茶匙 (15 ml)。

每日服4次。

DESCRIPTION:

SYRUPY Solution

Colour : Red

Flavour : Rose

CONTENT:

Each 5 ml contains:

Paracetamol 250 mg

(Preservative: Sodium Benzoate 0.1% w/v. Flavouring: Rose & Colouring: Carmoisine)

PHARMACODYNAMICS:

The mechanism of analgesic action has not been fully determined. Paracetamol may act predominantly by inhibiting prostaglandin synthesis in the central nervous system (CNS) and, to a lesser extent, through a peripheral action by blocking pain impulse generation. The peripheral action may also be due to inhibition of prostaglandin synthesis or to inhibition of the synthesis or actions of other substances that sensitise pain receptors to mechanical or chemical stimulation.

Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat regulating centre to produce peripheral vaso-dilation resulting in increased blood flow through the skin, sweating and heat loss. The central action probably involves inhibition of prostaglandin synthesis in the hypothalamus.

PHARMACOKINETICS:

Paracetamol is readily absorbed from the gastro-intestinal tract with peak plasma concentrations occurring about 30 minutes to 2 hours after ingestion. It is metabolised in the liver and excreted in the urine mainly as the glucuronide and sulphate conjugates. Less than 5% is excreted as unchanged Paracetamol. The elimination half-life varies from about 1 hour to 4 hours. Plasma protein binding is negligible at usual therapeutic concentrations but increases with increasing concentrations. A minor hydroxylated metabolite which is usually produced in very small amounts by mixed-function oxidases in the liver and which is usually detoxified by conjugation with liver glutathione may accumulate following Paracetamol overdose and cause liver damage.

INDICATIONS:

To relieve symptoms of fever, headache and pain.

RECOMMENDED DOSAGE:

6 to 12 years: 2 teaspoonfuls (10 ml).

Above 12 years and adults: 3 teaspoonfuls (15 ml).

To be taken 4 times daily.

CONTRAINDICATIONS:

Hypersensitivity to Paracetamol.

Hypersensitivity to hydroxybenzoates.

Patients with severe hepatic dysfunction.

For an accurate dosing, weigh your child and work out the dosage based on a 15 mg of Paracetamol for every kg body weight.

(Example: If your child weighs 20kg: $20 \times 15\text{mg} = 300\text{mg}$. $300\text{mg} / 250\text{mg} \times 5\text{ml} = 6\text{ml}$.)

Example dose by weight:

Body Weight (Kg)	Dose (ml)
20 - 23 Kg	6 ml
24 - 26 Kg	7 ml
27 - 29 Kg	8 ml
30 - 33 Kg	9 ml
34 - 39 Kg	10 ml
40 - 42 Kg	12 ml

- It is only for oral administration.
- Do not exceed the recommended dose.
- The lowest dose necessary to achieve efficacy should be used.
- Minimum dosing interval: 4 hours.
- Maximum duration of continued use without medical advice: 3 days.
- Not recommended for children under 6 years old.

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WARNING/PRECAUTIONS:

This drug should be used with care in hepatic and renal impairment. Long term usage should be avoided. Do not exceed the recommended dose or use for more than 10 days unless prescribed. Paracetamol should not be taken concomitantly with a drug that acts on the liver.

THIS PREPARATION CONTAINS PARACETAMOL.
DO NOT TAKE ANY OTHER PARACETAMOL CONTAINING MEDICINES AT THE SAME TIME.

Allergic alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away. Unsuitable for phenylketonurics.

DRUG INTERACTION:

The hepatotoxicity of Paracetamol, particularly after overdosage, may be increased by drugs which induce liver microsomal enzymes such as barbiturates, tricyclic antidepressants and alcohol.

Anion exchange resins: Cholestyramine reduces absorption of Paracetamol.

Anticoagulants: Prolonged regular use possibly enhances Warfarin.

Antivirals: Regular use of Paracetamol possibly reduces metabolism of Zidovudine (increased risk of neutropenia). Domperidone and Metoclopramide: Metoclopramide accelerates absorption of Paracetamol (enhanced effect).

PREGNANCY AND LACTATION:

There is epidemiological evidence of safety with Paracetamol in human pregnancy.

Paracetamol is excreted in breast milk, but not in clinically significant quantities.

SIDE EFFECTS/ADVERSE REACTIONS:

Adverse effects are usually mild. Nausea, gastro-intestinal upsets, rashes and allergic conditions have occurred. Overdose can cause hepatic damage, which is signified by jaundice, gastro-intestinal bleeding and hypoglycaemia. Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

SYMPTOMS AND TREATMENT OF OVERDOSE:

Although the likely toxicity of a Paracetamol overdose can be assessed by examining the changing pattern of Paracetamol blood concentrations, prompt specific treatment is essential. Any patient therefore, who has taken 7.5 g or more of Paracetamol and can be treated within 10 hours of ingestion should be given a specific antidote, even if the blood concentrations are not known. There is a risk that the suphadryl donors used as antidotes may exacerbate any liver damage if given 10 hours after the overdose. Once specific therapy is established then blood concentrations of Paracetamol can be monitored. Generally, treatment is required if the blood concentration is higher than a line (the '200' line) drawn on log/linear paper joining the points 200 mg per litre at 4 hours and 60 mg per litre at 10 hours. There have been suggestions that treatment may be necessary when lower concentrations of Paracetamol are present. Cysteamine is sometimes used by injection as an antidote but acetylcystein or methionine is now preferred. Acetylcystein is given by intravenous infusion in an initial dose of 150 mg per kg body-weight over 15 minutes followed by 50 mg per kg over 4 hours and then 100 mg per kg over the next 16 hours. Alternatively, methionine 2.5 g may be given by mouth every 4 hours to a total of 4 doses. Haemoperfusion may be worthwhile if too much time has elapsed since the poisoning to allow use of acetylcystein or methionine. Basic measures that may be required include dextrose and blood infusions. Removal of stomach contents by aspiration and lavage forms an early part of treatment and the administration of activated charcoal should be considered.

PACKING/PACK SIZE(S):

Amber glass bottle of 30 ml

**JAUHI UBAT DARIPADA KANAK-KANAK
KEEP OUT OF REACH OF CHILDREN
SHAKE WELL BEFORE USE**

Manufactured by:

DYNAPHARM (M) SDN. BHD.

198001011897 (65683-V)

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Product Registration Holder:

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